Patient ID	: QLD24/8676	Sample ID	: 24111281
Name	**: MR RAJ KUMAR**	Registration Date	: 28/11/2024 08:23 AM
Age/Gender	: 39 Yrs/Male	Report Date	: 28/11/2024 12:37 PM
Ref. Dr.	: SELF	QLD24/8676	
Center	: Q LABS DIAGNOSTICS		

HAEMATOLOGY REPORT

Test Description	Result	Unit	Biological Reference Ranges
COMPLETE BLOOD PICTURE			
Hemoglobin	13.0	gm/dL	13.0 - 17.0
RBC Count	4.70	mil/cu.mm	4.0 - 5.5
Hematocrit HCT	**38.7**	%	40 - 54
Platelet Count	**1.01**	lac/cmm	1.5 - 4.5
Red Cell Indices			
Mean Corp Volume MCV	**82.3**	fL	83 - 101
Mean Corp Hb MCH	27.7	pg	27 - 32
Mean Corp Hb Conc MCHC	33.6	gm/dL	31.5 - 34.5
RDW-CV	13.7	%	11.6 - 14.0
Total Count and DIFFERENTIAL LEUCOCYTE COUNT			
Total WBC Count	**3360**	cell/cu.mm	4000 - 10000
Neutrophils	51	%	40 - 75
Lymphocytes	40	%	20 - 40
Eosinophils	04	%	01 - 06
Monocytes	05	%	02 - 10
Basophils	00	%	00 - 02
Microscopic Blood Picture			
RBC Morphology	**Anisocytosis Microcytic Hypochromic**		
WBC Morphology	**Leucopenia**		
Platelet on smear	**Thrombocytopenia**		

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HBA1C/GLYCOCYLATED

HbA1c Glycosilated Haemoglobin	**6.1**	%	4 - 6

Method: HPLC, NGSP certified

Estimated Average Glucose : 128 mg/dL

As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Action suggested: > 8.0 Age < 19 years Goal of therapy: <7.5

ADA criteria for correlation

HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126
7	154
8	183
9	212
10	240
11	269
12	298

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled .
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments: HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations.

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BIOCHEMISTRY REPORT

Test Description	Result	Unit	Biological Reference Ranges
KIDNEY FUNCTION TEST(KFT)			
Serum Creatinine *Method: Modified Jaffe with no deproteinzation*	1.1	mg/dL	0.70 - 1.40
Urea *Method: Urease UV GLDH*	19.0	mg/dl	10.0 - 45.0
Blood Urea Nitrogen-BUN *Method: Calculated*	7.6	mg/dl	7 - 20
Uric Acid *Method: Uricase Peroxidase*	5.1	mg/dl	3.6 - 7.7
Sodium *Method: ISE*	140	mEq/L	135 - 146
Potassium *Method: ISE*	4.3	mEq/L	3.5 - 5.5
Chloride	101	mEq/L	96.0 - 106

Interpretation :

Kidney function tests help to screen the individual for renal disease and to determine the extent or progression of renal disease. These tests also aid in determining drug dosage for drugs excreted through the kidneys. The clinical syndrome resulting from decreased renal function and azotemia is called uremia Renal azotemia: glomerular nephritis and chronic pyelonephritis. Prerenal azotemia: severe dehydration, hemorrhagic shock, and excessive protein intake. Post renal azotemia: urethral stones or tumors and prostatic obstructions Measurement of urea in dialysis fluids is widely used in assessing the adequacy of renal replacement therapy.

In these prerenal situations, the plasma creatinine concentration may be normal.In obstructive post renal conditions, both plasma creatinine and urea concentrations will be increased, although there is often a greater increase in plasma urea than creatinine because of the increased back diffusion.These considerations give rise to the principal clinical utility of plasma urea, which lies in its measurement in conjunction with that of plasma creatinine and subsequent calculation of the urea nitrogen/creatinine ratio. This ratio has been used as a crude discriminator between prerenal and postrenal azotemia. Significantly lower ratios usually denote (1) acute tubular necrosis, (2) low protein intake, (3) starvation, or (4) severe liver disease (decreased urea synthesis). So even though blood urea is not an excellent marker of renal dysfunction as it rises quite late in the dysfunction and its rise is also not exclusive to kidney dysfunction, but for practical purposes serum urea level is still one of the most ordered test and forms an important part of the kidney function test.

Concentrations in excess of 6.0 mg/dL at 32 weeks gestation have been noted to be associated with a high perinatal mortality rate.

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LIVER FUNCTION TEST (LFT)

TOTAL BILIRUBIN *Method: Serum, Jendrassik Grof*	0.9	mg/dl	0.2 - 1.2
DIRECT BILIRUBIN *Method: Serum, Diazotization*	0.22	mg/dL	0.0 - 0.3
INDIRECT BILIRUBIN *Method: Serum, Calculated*	0.68	mg/dl	0.0 - 1.0
SGPT (ALT) *Method: Serum, UV with P5P, IFCC 37 degree*	21	U/L	0 - 49
SGOT (AST) *Method: Serum, UV with P5P, IFCC 37 degree*	**95**	U/L	0 - 46
ALKALINE PHOSPHATASE *Method: DGKC*	58	U/L	53 - 130
TOTAL PROTEIN *Method: Serum, Biuret, reagent blank end point*	6.4	g/dl	6.0 - 8.0
SERUM ALBUMIN *Method: Serum, Bromocresol green*	3.6	g/dl	3.2 - 4.6
SERUM GLOBULIN *Method: Serum, Calculated*	2.80	g/dl	1.8 - 3.6
A/G RATIO *Method: Serum, Calculated*	1.29		1.2 - 2.2

NOTE :

In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or Non alcoholic fatty liver disease, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

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COAGULATION STUDY REPORT

Test Description	Result	Unit	Biological Reference Ranges
PROTHROMBIN TIME (PTINR)			
Prothrombin Time for Patient's Plasma	**19.8**	Sec.	10 - 17
Prothrombin Time for control plasma	13.5	Sec.	
Prothombin Index	68.18	%	
International Normalised Ratio INR	**1.52**		0.8 - 1.5
ISI value of the Prothrombin used	1.1		
Prothombin Ratio	1.52		

Interpretation :

The prothrombin time (PT) and international normalized ratio (INR) are measures of the extrinsic pathway of coagulation.The INR is used only for patients on stable oral anticoagulant therapy. It makes no significant contribution to the diagnosis or treatment of patients whose PT is prolonged for other reasons.

Increased PT times may be due to:

Factor deficiencies(X , II , V , I), Coumadin (warfarin) therapy, Liver Diseases (Bile duct obstruction, Cirrhosis , Hepatitis), Hemmorhagic Disaease of the newborn, DIC, Malabsorption, Fibrinolysis, Vitamin K deficiency.

Interference in PT/INR Alcohol,antibiotics, aspirin, cimetidine,thrombin Inhibitors(Increase PT) Barbiturates, oral contraceptives, Hormone replacement therapy (HRT), and vitamin K (Decrease PT).

**** End of the report. ****
Please correlate clinically.

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